

Service Line Operating Model

Tallowbrook Health | Office of the COO | 2025-11-12 | v1.3

This document describes how Tallowbrook organizes clinical services into service lines that cross our two hospitals and four outpatient facilities, and how responsibility is divided between service line leaders and facility leaders. It replaces version 1.2 (March 2024). The model itself has not changed since July 2023; this revision clarifies decision rights for capital requests and aligns the reporting cadence with the monthly operating review.

Why a matrix

Most of our clinical programs are delivered in more than one building. An orthopedic patient may be seen at an outpatient site, operated on at one of three surgical sites and rehabilitated at the Pavilion or Stafford.

Managing each building separately left no single owner for the patient's path, the physician relationships or the program's growth.

The service line model gives each program an owner across sites while facility presidents stay accountable for the people, space and budgets inside their buildings.

We accept the overhead of a matrix because reorganizing staff around programs would split small Hospital Fort Bend departments into units too small to staff reliably.

The seven service lines and where they operate

We recognize seven service lines. Their footprints differ, and several services are deliberately offered at one site only.

- **Emergency & Hospital Medicine:** 24-hour emergency departments at both hospitals (38 treatment spaces at Medical Center West, 24 at Hospital Fort Bend) and hospitalist services.
- **Surgical Services & Orthopedics:** 10 operating rooms at Medical Center West, 6 at Hospital Fort Bend and the Surgery Center Grand Parkway.
- **Heart & Vascular:** cardiac catheterization at Medical Center West only; Hospital Fort Bend provides diagnostic cardiology and transfers STEMI patients to West by protocol.
- **Women's & Infant Services:** labor and delivery, Mother-Baby and the Level II NICU at Medical Center West only.
- **Imaging & Diagnostics:** hospital imaging at both campuses, outpatient imaging at the Pavilion and Stafford, and lab draw sites. Outpatient mammography sits here.
- **Rehabilitation & Therapy:** inpatient therapy at both hospitals and outpatient therapy at the Pavilion and Stafford.
- **Primary & Urgent Care:** Urgent & Family Care Mission Bend.

Dyad leadership and councils

Each service line is led by a physician leader and an administrative director working as a dyad.

The physician leader owns clinical standards, physician engagement and program development. The administrator owns operating plans, volumes and coordination with facility leaders.

Each line has a council that meets quarterly: the dyad, nursing directors of the units most involved, representatives from both hospitals and Decision Support.

Councils review volumes, access, quality measures and the quarterly contribution report, and they propose program changes. They do not approve budgets.

Decision rights: service line versus facility

The division of decision rights below is the part of the model most often misunderstood.

- Facility presidents decide staffing levels, hiring, unit assignments, space use and the facility operating budget.
- Service line dyads recommend program additions, equipment standards and physician recruitment priorities; the COO approves after the affected facility president reviews operating impact.
- Capital requests for program equipment are sponsored jointly: the service line sets clinical priority and the facility confirms space, utilities and staffing.
- Cross-site clinical protocols, such as STEMI transfers from Hospital Fort Bend, are owned by the service line and adopted by each medical staff.

Attribution and reporting rules

Encounters are attributed to one service line by primary service or principal procedure. An orthopedic surgery admission counts in Surgical Services & Orthopedics even if the patient spends a night in the ICU.

Emergency visits count in Emergency & Hospital Medicine unless the patient is admitted; the inpatient stay is then attributed by service.

Decision Support produces contribution margin by service line quarterly from the enterprise data warehouse, using the finance cost-allocation rules for overhead.

Contribution reports inform program decisions. They are not used in individual compensation.

Review cadence

Service line results appear in the monthly operating review on the third Thursday alongside facility results.

Councils meet quarterly, and each dyad presents an annual plan during the February-to-May budget cycle.

The board's Finance & Operations Committee sees service line results in its monthly review and receives a fuller service line report twice a year.

What the model does not cover

Tallowbrook has no inpatient behavioral health beds. Emergency psychiatric consultation comes from a tele-psychiatry partner, and patients needing admission are transferred to area behavioral health hospitals.

Behavioral health is managed within Emergency & Hospital Medicine rather than as a separate line.

Neither hospital is a Level I or II trauma center; major trauma is transferred to regional centers.

The Surgery Center Grand Parkway takes part in the surgical council but is governed by its own joint venture board.